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Badgett RG. Emberson J. A summary meta-narrative from positive deviance and similar qualitative studies that contrast clinician styles stratified by clinical sites with positive versus other outcomes. [add date of your download]. Available at https://ebmgt.github.io/clinician_culture/ .

Table. A summary meta-narrative from positive deviance and similar qualitative studies that contrast clinician styles stratified by clinical sites with positive versus non-positive outcomes*.

Setting	Positive outcomes	Non-positive outcomes
Collaborative work	Promotion, and advocacy	
	“Passion on the part of physician leaders to continually hit that mark and for the best outcomes...” ¹	“Physician presence in championing...quality improvement efforts was weak” ¹
	“medical staff organization factors as involvement of the medical staff president with the hospital governing board, overall physician participation in hospital decision making, ... are positively associated with higher quality-of-care” ²	“[T]here’s not enough physician leadership on the committee” ¹ “You should remember: I don’t care about any patients but mine”³
	Meeting	
	“frequency of medical staff committee meetings ... are positively associated with higher quality-of-care” ²	“It's just recently that we're able to find out which physicians are not following the guidelines... and we're finding that it's our [private] hospitalists that are more of the culprit. And those are the hardest group that we have not been able to get into a room to have conversations with... they don’t have regular meetings. ” ⁴
	Group decision making	
Clinical work	“Physicians and non-physicians alike commented on the levelling effect of working together in the coalition, with more equitable participation and engagement among members, who grew more unified as a team. In one hospital, the coalition set a new tone for risk-taking and working on the ‘leading edge’, even if some ideas were not successful.” ⁵ “Our physician champion, has been much more willing to say, ‘I don’t know,’ and rely on other people, which is something that I don’t think he necessarily did a while back.” ⁵	“There is still this deference to authority...we tend to put our physicians up there...‘our physician said it should be, so it should be.’” ⁵ “Opportunities for creativity were constrained by deference to hierarchical relationships; non-physician staff yielded too readily to physicians and physicians showed limited respect for diverse expertise.” ⁵

	“...Nurses know that they are 100% supported, all the way up to the top of the organization, that they are empowered to call rapids regardless if they’re being told not to call a rapid [response]...”⁶	“...A lot of them are afraid to call the physician. So sometimes the physician would be angry that they called a rapid response...”⁶ “Truthfully, I’m not going to lie. There are times where I see stuff that’s wrong and I’m just like, forget it... if it’s not going to hurt anybody...if it was a couple of [antibiotic] days.”⁴ “You know...one of them when I call, gets very angry and seems quite put out that I am talking to [them] in the first place.”⁴ “I gave you orders, and what are you calling me again for?”¹
	Collaboration on clinical care	
	“Clinicians frequently discuss difficult cases to solicit the opinions and insights of their colleagues.”⁷	“Providers...tended to practice without the benefit of their colleagues’ opinions.”⁷

Notes:

* In addition to clinical outcomes, other outcomes include measures of team performance in the study by Hu³ and tactics previously associated with clinical outcomes at the organizational level by Curry⁵.

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